

MIG-11

Migrant Interview: Inclusion and Xenophobia

Interviewee Profile

Interview code: MIG-11

Age: 29

Gender: Female

Country of origin: Kenya

Host country: United States

Years in host country: 4

Occupation: Registered Nurse

Interview

Q1. Can you tell me about your journey and what brought you to your host country?

A: I moved to the United States four years ago through an international nursing recruitment programme. There was a serious shortage of nurses here, and the pay and the professional opportunities on offer far exceeded anything I could realistically earn or achieve back home in Kenya. I wanted to advance my career, to develop my skills in a well-resourced healthcare system, and to be able to support my family financially through remittances, which make a real difference back home. So I took the leap into the unknown, leaving behind everything familiar to start over in a country I knew mostly from television and imagination. The reality of arriving, of being suddenly alone in an enormous and unfamiliar society, was both exhilarating and deeply frightening in equal measure.

Q2. What does 'inclusion' mean to you personally?

A: Inclusion means that my patients trust my care regardless of my accent or the colour of my skin, that they see the competent, trained professional in front of them rather than reacting to my foreignness. It means that my colleagues treat me as a genuine equal, a fellow nurse rather than a lesser one, and that I am able to advance in my career without discrimination quietly holding me back from the opportunities I have earned. True inclusion, as I have come to understand it, is when my Kenyan background is regarded as something that enriches the workplace and brings a valuable perspective, rather than as a deficit or an obstacle that I must constantly work to overcome in the eyes of others.

Q3. Have you experienced xenophobia or discrimination? Can you describe it?

A: Yes, I have faced both xenophobia and racism, and the two are often intertwined in my experience, sometimes from patients who openly question my competence the moment they hear my accent, or who make pointed comments about where I must be from. A few patients have even requested a different, 'American' nurse, refusing my care outright, which is disheartening after all my training and dedication. Being simultaneously an immigrant and a Black woman means that I face a kind of compounded, layered prejudice in certain situations, where I cannot always tell which aspect of who I am is prompting the hostility, only that I am being treated as somehow less trustworthy or less capable than I know myself to be.

Q4. What have been the biggest challenges in adapting to your new country?

A: The biggest challenges were adjusting to the significant differences in the American healthcare system, which operates quite differently from what I trained in, and adapting to the fast-paced, high-pressure, and notably litigious environment, where the fear of lawsuits shapes so much of how care is delivered. The cost of living and the expense of the additional courses and examinations required to fully validate and upgrade my credentials here were a real financial burden. And being so far from my family and my entire support system, navigating everything alone in a vast new country without the network of relatives I had always relied upon, was emotionally and practically very difficult, especially in the beginning.

Q5. How would you describe your experience in the workplace or professional environment?

A: My hospital colleagues are, on the whole, genuinely supportive, and the nursing profession attracts a great many immigrants, so I am far from alone in my situation, which makes an enormous difference. There is a strong and welcoming contingent of African and Filipino nurses in particular who form a supportive community, sharing advice and understanding and looking out for one another. Most of my patients appreciate the care I provide once they experience it, and the many positive relationships I build with patients and colleagues alike substantially outweigh the occasional hostile or prejudiced encounter, which, while painful, is not the norm and does not define my working life or my sense of my own worth.

Q6. How do you balance maintaining your cultural identity with integrating into the host society?

A: I stay connected to my Kenyan identity through my faith, which is central to my life, through cooking Kenyan dishes that remind me of home, through speaking Swahili with friends from back home, and through sending remittances that keep me tied to my family and their wellbeing. I have embraced many aspects of American life and its opportunities while carefully keeping the cultural values I was raised with, particularly the deep emphasis on community and on respect for one's elders, which I never want to lose. I feel proudly and unmistakably Kenyan while building a meaningful and productive life here in America, and I do not experience these two things as being in any real tension with each other.

Q7. How do you feel the media portrays migrants from your background?

A: American media too often portrays Africa as a single undifferentiated monolith of poverty, disease, and conflict, an image that directly shapes how many people perceive me before they know anything real about me. People are frequently surprised to discover that I am well-educated and professionally trained, as though that contradicted their expectations of where I come from. Positive, nuanced, and accurate representation of Africans and of the diversity and progress of the continent is rare in the media I encounter here. This widespread ignorance means that I constantly find myself having to counter assumptions about my background, my country, and my abilities, correcting a picture that was painted long before I arrived.

Q8. Have integration programs or community support been helpful in your experience?

A: The recruitment programme that brought me here provided some initial orientation, which was helpful for getting started, but the integration support beyond the job itself was quite limited, and I

largely navigated housing, banking, transportation, and the broader cultural adjustment on my own, figuring things out through trial and error. More comprehensive and sustained support for internationally recruited healthcare workers, who are filling genuine and critical shortages, would ease the transition considerably and would allow us to focus our energy more fully on the demanding and important clinical work we came here to do, rather than being distracted by the constant stress of sorting out the basics of a new life alone.

Q9. How has the next generation (or younger migrants) adapted compared to you?

A: I do not have children yet, but I hope very much that if and when I do, they will grow up proud of their Kenyan heritage while also thriving fully as Americans, holding both identities without conflict or shame. I have watched the children of other African immigrants excel academically and professionally here, achieving remarkable things, even as they navigate the realities of racism and of questions about their belonging. My hope is that the next generation will inherit a more accepting and less prejudiced society than the one that currently exists, one in which their background is simply a part of who they are rather than something they must constantly explain or defend.

Q10. In your opinion, what would help reduce xenophobia in society?

A: To reduce xenophobia, education about the true diversity of Africa and about the enormous contributions that immigrants make to American healthcare in particular is essential; we quite literally keep hospitals staffed and functioning, and that reality deserves to be widely known. The media must show fuller, more accurate, and more varied portrayals of Africans and immigrants generally, moving beyond the tired and damaging stereotypes. And personal contact matters enormously, as it always does; when patients experience excellent, compassionate care from an immigrant nurse, their prejudices very often soften and give way. Our shared humanity has a way of emerging through genuine human connection, which is why direct experience changes minds far more than argument ever could.

Q11. What are your hopes and dreams for the future?

A: I hope to specialise further in my field, perhaps by becoming a nurse practitioner, and to advance into positions of clinical leadership where I can have a greater impact. I want to support other internationally recruited nurses in navigating the same system and challenges that I faced, offering the guidance that I largely had to do without. And looking further ahead, I dream of investing in healthcare projects back in Kenya, using the advanced knowledge, skills, and experience I have gained here to give something concrete back to the community and the country that raised me and made me who I am, closing the circle in a meaningful way.

Q12. What advice would you give to new migrants arriving in your host country?

A: My advice to new migrants is to be genuinely confident in your skills and your training, and never to let anyone else's prejudice make you doubt yourself or your worth, because that self-doubt is the most damaging thing you can absorb. Build a support network of fellow immigrants and of open-minded locals, because you will need both understanding and connection. Take the time to understand your professional environment thoroughly, including its unwritten rules and expectations. Stay connected to home for the emotional strength it provides. And let your excellent, dedicated work speak for itself and challenge every stereotype that people might carry, because in the end that is the most powerful answer

to prejudice.